



Patient Name: **PRATHAPA CHANDRA VARMA**

Lab No. : MBLC3843
Sex/Age : 70 Years / Male
Reg. Date : 04-May-2023

Referred By : Dr.P N VARMA
Sample Collected Time: 04-May-2023 09:11 am
Auth. Date & Time : 04-May-2023 12:58 pm
Report Printed on : 04-May-2023 01:26 pm

Test	Method	Observed Value	Units	Biological Reference Range	Sample
COMPLETE HAEMOGRAM (Automated blood cell counter)					
HAEMOGLOBIN (HB)	Cyanide-free SLS-haemogl obin	12.4	gm/dL	Birth :18.0 +/-4.0, Day 3:18.0+/-3.0, 1 month:14.0+/-2.5, 2 months 11.2+/-1.8, 3 months - 6 years 12.6 +/- 1.5, 6 - 12 years:13.5+/-2.0., Adults : Male:13.0 -16.0; Female:12.0-15.0 g/dl	EDTA
TOTAL WBC COUNT (TC)	Flow Cytometry	8,070	/cumm	Upto1 year : 6000-16000 2-6 Years :5000-15000 6-12 Years :5000-13000 Adults : 4000-10000	EDTA
DIFFERENTIAL COUNT (DC)					
NEUTROPHILS	Flow Cytometry	65.4	%	Upto 1 year : 17-47% 2 - 6 years : 15- 65 % 6 - 12 year : 35 - 70 % Adults: 40-80%	EDTA
LYMPHOCYTES	Flow Cytometry	23.8	%	Upto 6 year : 45-75% 6 - 12 year : 25- 54 % Adults: 20-40%	EDTA
EOSINOPHILS	Flow Cytometry	5.7	%	Upto 12 year : 0 - 6 % Adults : 0 - 6 %	EDTA
MONOCYTES	Flow Cytometry	4.5	%	Upto 1 year : 0-8% 2 - 6 years : 0- 7 % 6 - 12 year : 0 - 6 % Adults: 0-10 %	EDTA
BASOPHILS	Flow Cytometry	0.6	%	0-2 %	EDTA
RBC	Hydrodynami c focusing DC detection	4.26	mill/cu mm	Upto1 year : 3.9-5.1 2 - 12 years : 4.0-5.2 Adult male :4.5-5.5 Adult female : 3.8-4.8	EDTA
PLATELET COUNT	Hydrodynami c focusing DC detection	2.56 Lakhs/Cumm		Upto 1 year : 2.0 - 5.5 Lakhs/cumm 2-6 years :2.0- 4.9 Lakhs/cumm 6-12 Years - 1.7-4.5 Lakhs/cumm adults 1.5 - 4.1 Lakhs/cumm	EDTA
PACKED CELL VOLUME (PCV)	Flow Cytometry	37.9	%	Upto1 year : 30-38 2-6 years :34-40 6-12 Years - 35-45 Adults male :40-50 Adults Female:36-46	EDTA
MCV	Derived Parameter	89.0	cubic/ micro	Upto1 year : 72-84 2-6 years : 75-87 6-12 years :77-95 Adults :83-101	EDTA
MCH	Calculated Parameter	29.1	pico gram	Upto 1 Year : 25-29 2-6 Years :24-30 6-12 Years :25-33 Adults :27-32	EDTA





Patient Name: **PRATHAPA CHANDRA VARMA**

Lab No. : MBLC3843
Sex/Age : 70 Years / Male
Reg. Date : 04-May-2023

Referred By : Dr.P N VARMA
Sample Collected Time: 04-May-2023 09:11 am
Auth. Date & Time : 04-May-2023 12:58 pm
Report Printed on : 04-May-2023 01:26 pm

Test	Method	Observed Value	Units	Biological Reference Range	Sample
MCHC	Calculated Parameter	32.7	%	Upto 1 Year : 32-36 2-6 Years :31-37 6-12 Years :31-37 Adults : 31.5-34.5	EDTA
LYM-TC	Flow Cytometry	1,920	/cumm	Upto 1 years [3500 - 11,000] 2 - 6 years [6000 - 9,000] 6- 12 years [1000 - 5,000] Adult [1000 - 3,000]	EDTA
NEUT-TC	Flow Cytometry	5,280	/cumm	Upto 1 years [1000 - 7,000] 2 - 6 years [1,500 - 8,000] 6- 12 years [2,000 - 8,000] Adult [2000 - 7,000]	EDTA
MONO-TC	Flow Cytometry	360		[0-1000]	EDTA
EOS-TC	Flow Cytometry	460	/cumm	Upto 12 years [0 - 1,000] Adult [0 - 500]	EDTA
RDW-SD	Derived Parameter	41.8	FL	[39-46]	EDTA
RDW-CV	Derived Parameter	12.8		[11.6-14.0]	EDTA
MPV	Derived Parameter	9.9	FL	[8.5 - 13.3]	EDTA
BASO-TC	Flow Cytometry	50		[0-100]	EDTA
PDW		10.9	FL	9 -14 FL	EDTA
P-LCR		23.7	%	15 - 35 %	EDTA
PCT		0.25	%	0.15 - 0.62	EDTA

Technology used : Sysmex KX-21 (An automatic multi parameter blood cell counter)

----- End of HAEMATOLOGY Report -----

Reviewed by:

Mrs.PREETHA SASIKUMAR
(CMAI DMLT)



Authorized by:


DR.ELIZABETH GEORGE
M.B.B.S,MD Pathology





Patient Name: **PRATHAPA CHANDRA VARMA**

Lab No. : MBLC3843
Sex/Age : 70 Years / Male
Reg. Date : 04-May-2023

Referred By : Dr.P N VARMA
Sample Collected Time: 04-May-2023 09:11 am
Auth. Date & Time : 04-May-2023 12:58 pm
Report Printed on : 04-May-2023 01:26 pm

Test	Method	Observed Value	Units	Biological Reference Range	Sample
COMPLETE HAEMOGRAM (Automated blood cell counter)					
ERYTHROCYTE SEDIMENTATION RATE (ESR)	Westergren Method	45	mm/hr	Male 0-10 mm/hr Female 0-20 mm/hr	EDTA
Technology used : Sysmex KX-21 (An automatic multi parameter blood cell counter)					
----- End of HAEMATOLOGY Report -----					
Reviewed by:			Authorized by:		
Mrs.PREETHA SASIKUMAR					
(CMAI DMLT)			DR.ELIZABETH GEORGE M.B.B.S,MD Pathology		

Page 3 of 5

- Fully Automated Laboratory
- Digital X-RAY
- Digital ECG
- Color Doppler Ultrasound
- Echocardiogram
- Treadmill Test
- Mammography
- Pulmonary Function Test
- NCV
- EEG

LAB DIRECTOR: **Dr. Elizabeth George**, MBBS, MD CONSULTANT MICROBIOLOGIST: **Dr. Asha Mathew**, MBBS, MD.
CONSULTANT RADIOLOGIST: **Dr. Sreedevi Varun**, MBBS, DMRD, DNB CHIEF TECHNICIAN: **Mrs. Valsa Varghese**, CMAI.



Near International Stadium
Metro Pillar No : 548, Palarivattom
Kochi - 25, Kerala

Ph : 0484- 2 333 888
e-mail : enquire@hitechcochin.com
www.hitechdiagnosticcentre.com

OPEN ALL DAYS
5 AM - 10 PM





Patient Name: **PRATHAPA CHANDRA VARMA**

Lab No. : MBLC3843
Sex/Age : 70 Years / Male
Reg. Date : 04-May-2023

Referred By : Dr.P N VARMA
Sample Collected Time: 04-May-2023 09:11 am
Auth. Date & Time : 04-May-2023 12:57 pm
Report Printed on : 04-May-2023 01:26 pm

Test	Method	Observed Value	Units	Biological Reference Range	Sample
UREA	Urease,dye	26	mg/dL	[10 - 45]	SERUM
Technology used : Samples tested on VITROS - Drychemistry Analyzers with intellicheck technology					
NOTE: A high serum urea level occurs in acute and chronic renal failure, pyelonephritis, urinary tract obstruction and shock. High levels are also seen in severe congestive heart failure, hyperalimentation, diabetic ketoacidosis, tissue damage associated with trauma, dehydration and bleeding from the gastrointestinal tract. Low levels occurs in pregnancy, with decreased protein intake, acute liver failure and with intravenous fluid administration.					
CREATININE	Enzymatic	0.9	mg/dL	Newborn (1-7 days) :0.6 - 1.1 Infant (8-31 days) : 0.3 - 0.7 Children (31 days-1yr) : 0.4 - 0.7 Children (1 yr - 14 yrs) : 0.2 - 0.7 Adult: Male : 0.7 - 1.5 Female : 0.6 - 1.5	SERUM
Technology used : Samples tested on VITROS - Drychemistry Analyzers with intellicheck technology					
NOTE: Several factors may influence serum creatinine independent of changes in GFR. Decreased levels are seen in individuals with small stature, elderly persons with decreased muscle mass, cachexia, amputation or muscle diseases and pregnancy. Because some renal clearance of creatinine is due to tubular secretion, drugs that inhibit this secretory component (eg, cimetidine and trimethoprim) may cause small increases in serum creatinine without an actual decrease in GFR. Patients on long term lidocaine therapy may show an increase in serum creatinine levels.					
<u>ELECTROLYTES</u>					
SODIUM	ISE direct	140	mEq/L	[136 - 145]	SERUM
POTASSIUM	ISE direct	4.9	mEq/L	3.5 - 5.1	SERUM
CHLORIDE	ISE direct	102	mEq/L	[96 - 108]	SERUM
----- End of BIOCHEMISTRY Report -----					
Reviewed by:		 MC-2293		Authorized by:	
MARYKUTTY JOSE (CMAI,DMLT)				 DR.ELIZABETH GEORGE M.B.B.S,MD Pathology	

Page 4 of 5

- Fully Automated Laboratory
- Digital X-RAY
- Digital ECG
- Color Doppler Ultrasound
- Echocardiogram
- Treadmill Test
- Mammography
- Pulmonary Function Test
- NCV
- EEG

LAB DIRECTOR: **Dr. Elizabeth George**, MBBS, MD CONSULTANT MICROBIOLOGIST : **Dr. Asha Mathew**, MBBS, MD.
CONSULTANT RADIOLOGIST: **Dr. Sreedevi Varun**, MBBS, DMRD, DNB CHIEF TECHNICIAN : **Mrs. Valsa Varghese**, CMAI.



Near International Stadium
Metro Pillar No : 548, Palarivattom
Kochi - 25, Kerala

Ph : 0484- 2 333 888
e-mail : enquire@hitechcochin.com
www.hitechdiagnosticcentre.com

OPEN ALL DAYS
5 AM - 10 PM





Patient Name: **PRATHAPA CHANDRA VARMA**

Lab No. : MBLC3843
Sex/Age : 70 Years / Male
Reg. Date : 04-May-2023

Referred By : Dr.P N VARMA
Sample Collected Time: 04-May-2023 09:11 am
Auth. Date & Time : 04-May-2023 12:57 pm
Report Printed on : 04-May-2023 01:26 pm

Test	Method	Observed Value	Units	Biological Reference Range	Sample
ELECTROLYTES					
BICARBONATE	Enzymatic	29	mEq/L	[22 - 30]	SERUM
C-REACTIVE PROTEIN (CRP)*	Nephelometry	5.0	mg/L	< 10	SERUM

Technology used : Samples tested on VITROS - Drychemistry Analyzers with intellicheck technology

Comments

1.CRP is an acute phase reactant which is used in inflammatory disorders for monitoring course and effect of therapy. It is most useful as an indicator of activity in Rheumatoid arthritis, Rheumatic fever, tissue injury or necrosis and infections.

2.As compared to ESR, CRP shows an earlier rise in inflammatory disorders which begins in 4-6 hrs, the intensity of the rise being higher than ESR and the recovery being earlier than ESR.

3.Unlike ESR, CRP levels are not influenced by hematologic conditions like Anemia, Polycythemia etc

----- End of BIOCHEMISTRY Report -----

Reviewed by:

MARYKUTTY JOSE
(CMAI,DMLT)



Authorized by:


DR.ELIZABETH GEORGE
M.B.B.S,MD Pathology

